SACTCHECK · REGIMEN-SPECIFIC SACT CONSENT

Carboplatin AUC 1.5 with radiotherapy

Head and neck cancer: weekly chemoradiation · NCCP 00332 · version 4 · 7-day cycle

PROTOTYPE · GOVERNANCE REVIEW REQUIRED

Patient name	DOB	Hospital no.	Date

Treatment context
Carboplatin and radiotherapy are used together as part of the treatment plan for head and neck cancer. The team explains the treatment aim, expected benefit and why this medicine and schedule suit your situation.

Schedule
Carboplatin AUC 1.5 is given by IV infusion over 30 minutes once every 7 days for 7 planned weekly doses, only while radiotherapy is being given. It must be stopped if the radiation course is cut short. Radiation sessions follow their own calendar.

1. TREATMENT DISCUSSION

- ☐ Purpose, expected benefit and alternatives explained
- ☐ Indication, medicines, sequence and duration confirmed
- ☐ Blood tests and any individually indicated heart checks discussed
- ☐ Support medicines and phase changes explained
- ☐ Fertility, contraception and breastfeeding discussed where relevant
- ☐ Common, serious and lasting drug-specific risks discussed below
- ☐ Urgent symptoms and local contact details confirmed

2. REGIMEN / AGENT-SPECIFIC MATERIAL RISKS

Carboplatin
PLATINUM CHEMOTHERAPY

- ☐ Low blood counts and serious infection
- ☐ Low platelets and bleeding
- ☐ Anaemia and fatigue
- ☐ Nausea and vomiting
- ☐ Infusion or allergic reactions
- ☐ Kidney, nerve or hearing effects

Radiotherapy
LOCAL TREATMENT

- ☐ Sore mouth and throat
- ☐ Dry mouth, taste and saliva changes
- ☐ Swallowing or voice problems
- ☐ Skin reaction in the treated area
- ☐ Jaw stiffness and dental problems
- ☐ Some swallowing, saliva or tissue effects can be lasting

Discuss the individual aim, expected benefit, alternatives and uncertainties, including why carboplatin is chosen and how it is coordinated with radiation. Explain low blood counts, infection and sepsis, bleeding, anaemia, nausea, infusion allergy and possible kidney, hearing or nerve effects; fertility and pregnancy; and the appointment burden. Radiation needs a separate discussion of the treated area, mouth and skin effects, swallowing, dental and saliva changes, and possible lasting effects. Confirm any earlier or later treatment separately.

Individual aim / benefit, alternatives, material risks and questions:

3. CONSENT CONFIRMATION

- ☐ Expected benefit, important risks and reasonable alternatives discussed
- ☐ Patient questions invited and answered; patient-specific material risks / priorities considered
- ☐ Emergency contact / toxicity advice and written information offered where appropriate
- ☐ Patient understands consent is voluntary and may be withdrawn

Patient / person giving consent _____ Date _____

Clinician obtaining consent _____ Date _____

Prototype only. This one-page form is a regimen-specific consent aid and requires formal governance, consultant and oncology-pharmacy review before clinical use. The current NCCP regimen, local/national consent policy, current medicine information, patient-specific material risks and clinical judgement remain authoritative.
Base regimen: NCCP 00332 v4.

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